



Public Health Guidelines for Social Connection: An International Delphi Study

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ABSTRACT

Background: Loneliness and social isolation are linked to increased morbidity and mortality, with health risks comparable to sedentary lifestyles and poor nutrition. Promoting social connection has therefore become a global public health priority. Evidence-based guidelines could help raise awareness of the health impacts of disconnection, shape individual behaviors, and inform community programs and policies.

Objective: This study aimed to develop recommended public health guidelines for social connection using a Delphi methodology.

Methods: Experts from diverse subfields of Psychology were recruited through networks, nominations, and targeted invitations. In Round 1, open-text responses identified key principles and potential guidelines for individuals and communities. A grounded thematic analysis synthesized these responses into draft guidelines. In subsequent rounds, experts rated their support for each statement. Guidelines with $\geq 80\%$ expert endorsement were refined through focus groups.

Results: The final product included 12 guidelines: six for individuals and six for communities. Community guidelines focus on: (1) raising awareness of connection, (2) supporting social-emotional development, (3) prioritizing social health in policy, (4) designing connection-friendly environments, (5) promoting accessibility and inclusion, and (6) measuring social wellbeing. Individual guidelines emphasize: (1) making connection a lifelong priority, (2) cultivating a positive social outlook, (3) building diverse networks, (4) prioritizing meaningful interactions, (5) developing new relationships, and (6) using technology wisely.

Conclusions: These expert-informed guidelines may serve as a framework for advancing individual and population-level efforts to strengthen social wellbeing.

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RESEARCH-IN-CONTEXT

(1) What is already known about the topic?

Loneliness and social isolation are widely recognized as serious public health risks, increasing the likelihood of morbidity and mortality. Despite this, public health systems have historically lacked formal, evidence-based guidelines for promoting social connection, leaving a critical gap in prevention efforts and population health strategies.

(2) What does this study add to the literature?

This is the first international Delphi study to produce expert-informed public health guidelines for social connection. Drawing from a multidisciplinary panel, the study identifies 12 endorsed guidelines—six for individuals and six for communities—offering a clear, actionable framework grounded in empirical research and professional consensus across psychology, public health, and community development.

(3) What are the policy implications?

These guidelines provide a foundation for integrating social connection into health promotion, service design, and policy-making. By recognizing social wellbeing as essential to public health—on par with physical activity, nutrition, and sleep—governments, organizations, and communities can implement targeted, inclusive strategies that address isolation, foster connection, and improve population-level health outcomes.

1. Background

Empirical evidence has linked loneliness and social isolation to increased morbidity and mortality. A meta-analysis of 90 cohort studies indicated that an approximately 30 % increased risk of all-cause mortality was associated with social isolation and a 14 % increased risk from loneliness [1]. Other measures of social connectedness demonstrate larger effects, with social integration measures (e.g., the presence of high quality relationships) indicating a 91 % increased odds in survival [2]. These estimates place loneliness and social isolation on par with traditional health risk factors, including poor diet, sedentary living, alcohol use, and smoking [3].

For decades, researchers and scholars have recognized the importance of social connection. Early work in suicide (1897) by Emile Durkheim investigated the link between social integration and suicide death [4]. Epidemiological studies since the mid-20th century consistently highlight the health risks of social disconnection among bachelors [5], war wives [6], and widows [7] and over the last several decades populations have spent less and less time with their increasingly small social networks – resulting in a so called “disconnection crisis” [8,9]. Acknowledging this, the World Health Organization (WHO) defined health as “a state of complete physical, mental and *social* well-being” (emphasis added) – incorporating belonging and connection as core components for conceptualizing health within public health practice [10]. Yet, although physical and mental health have drawn widespread attention in the design of public health systems, social wellbeing has only recently garnered public health policy attention [11]. Recent innovations in public health have raised awareness about social wellbeing – particularly through a lens of crisis, with governments and organizations acknowledging an emergent “epidemic of loneliness.” The United Kingdom’s appointment of a “Minister of Loneliness,” followed by Japan’s, highlights the mounting interest in promoting social wellbeing [12]. The COVID-19 pandemic added fuel to the fire – leading to the United States Surgeon General publishing and actively promoting an advisory on the importance of social connection and community in 2023¹³ and, in that same year, the WHO establishing a Commission on Social Connection [14].

To advance social connection as a public health priority, researchers have called for the development of public health guidelines for social connection, to augment guidelines on nutrition, exercise, and other widespread public health concerns (e.g., alcohol, marijuana). For example, the present project was funded by the Canadian Government in 2022 in response to the COVID-19 pandemic. Similarly, Holt-Lunstad (2023) recently argued for the creation of evidence-based guidelines that provide information regarding the optimal size of social networks, frequency of social contact, diversity of relationship types, mode and context of interaction, and quality of relationships [15].

We have argued that public health guidelines for social connection such as these could support individual, programmatic, and policy efforts to promote social connection and wellbeing by (a) raising awareness about the connections between social connection and health, (b) encouraging the development and implementation of policies and sustained programs that assist individuals in achieving these targets, (c) educating individuals about how to achieve optimal social connection and wellbeing, (d) promoting healthy social development and behaviours (e) catalyzing applied research on social connection and health, and (f) providing clear measurable outcomes for social connection that can be used in program evaluation, public health monitoring, and academic research [16]. The aim of the present study was to develop a set of recommended public health guidelines for social connection using a Delphi study approach.

2. Methods

2.1. Study design and rationale

Delphi Studies are an approach to consensus building through iterative surveys with an expert advisory panel [17]. This methodology was selected because of the complexity of social wellbeing and the nature of empirical evidence related to this topic [17]. Specifically, loneliness, social isolation, and social connection are nuanced phenomena spanning multiple disciplines, including social psychology, sociology, and public health. These epistemological and ontological complexities create significant challenges in the research, including issues related to measurement and study design [18]. Furthermore, studies are rarely designed in ways that can be used for guideline development. Similar challenges plague research on physical activity, nutrition, and alcohol use. Given all of these factors and considerations, we argue that expert-based development of social connection guidelines provides the most suitable path for developing an initial set of much-needed guidelines.

2.2. Participants

We recruited self-identified experts in loneliness, social isolation, social connection, and community from: (a) the corresponding author emails associated with frequently-cited authors studying loneliness, isolation, and connection, (b) list servers of professional psychological research societies, and (c) snowball sampling of these networks by either peer nomination or distribution of our invitation email. Between 500 and 700 invitations were directly extended by members of the research team to authors and their professional networks. Additionally, we sent invitations to four email list servers associated with leading psychology professional organizations and networks (e.g., Society for Personality and Social Psychology, International Association for Relationship Research, Global Initiative on Loneliness and Connection).

2.3. Procedures and analysis

All study participants provided informed consent prior to participation. The study protocol was reviewed by the research Ethics Board at Simon Fraser University (Protocol #30001448). After providing informed consent, participants were invited to four structured rounds of

consultation. Briefly, Round 1 served as an open exploratory phase, where participants freely brainstormed principles and potential guidelines without being presented with any draft framework. Round 2, assessed the support for statements drafted from Round 1 data. After review of Round 2 results, draft guidelines were developed and subsequently introduced for consensus ratings in Round 3. After Round 3, wording changes were made through an iterative series of focus groups. Additional information about each round of data collection is provided below.

- Round 1 ($n = 94$). Open-text questions were used to brainstorm: (a) general principles that would be important for the development and implementation of public health guidelines for social connection, (b) potential guidelines tailored for individuals, (c) potential guidelines tailored to the group level (e.g., organizations, communities, governments), and (d) factors that shape social connection and well-being. Leveraging the data collected through these open-text responses, we conducted a grounded thematic analysis, inductively identifying and synthesizing ideas within each section in order to develop and categorize potential guidelines that could be used for Round 2.
- Round 2 ($n = 84$). We presented individual guidelines to expert consultants and asked them to rate: (a) their level of agreement with the principles for guideline development (Strongly Agree [4], Agree [3], Neither Agree Nor Disagree [2], Disagree [1], Strongly Disagree [0]), (b) their assessment of the importance of each guideline (Absolutely essential [5], Very important [4], Moderately important [3], Slightly important [2], Unimportant [1], Should NOT be included [0]) and (c) the importance of each identified factor associated with social connection (Extremely [4], Very [3], Moderately [2], Slightly [1], Not at all [0]). Participants also had the opportunity to provide further open text comments at each section. Descriptive statistics were used to quantify levels of support, and thematic analyses were based on interpretations of the open text data.
- Round 3 ($n = 82$). We presented draft recommended guidelines which were developed based on the results from Round 2. These guidelines were created by a grounded thematic analysis balanced against ratings of importance provided by reviewers in order to achieve a sufficiently concise set of guidelines. For each guideline, participants rated their level of expertise with respect to the content of the guideline ([1] High, [2] Moderate, [3] Low, [4] None), were asked to indicate whether they did or did not support the inclusion of the guideline ([1] Yes, [2] No, [3] I prefer not to vote on this guideline), and asked to provide feedback regarding the guideline. A *priori* pass-fail criteria were established for this exercise, requiring endorsement for inclusion by 80 % of those who selected either “Yes” or “No”; 75 % of all raters (including those who preferred not to vote on the guideline); and 85 % of those who indicated that they had “High” Self-Rated domain knowledge of the guideline. The cut-off criteria (i.e., 75 % of all participants and 85 % of domain experts) were included because some raters who might partially object to a guideline selected “prefer not to vote.” As well, given the diversity of expertise among raters and the breadth of topics covered by the guidelines, we wanted to make sure that among those with relevant expertise, each guideline would be deemed appropriate.
- Round 4 ($n = 35$). We conducted a series of focus group interviews to further refine the wording of the guidelines. Refinement was done through discussion and incorporating suggestions iteratively across groups. As the guidelines had already been approved by consensus vote in Round 3, it was our intention to remain true to the original spirit of the guidelines. However, we also acknowledged that refinement was possible – particularly to effectively communicate the guidelines to public audiences. From these guidelines we

developed final recommendations for social connection and additional considerations. The revision of the guidelines was iterative, taking place between each focus group interview (i.e., 7 total revisions). All final guidelines and associated additional considerations were adjudicated by our internal research team.

Importantly, the guideline development process included activities such as evidence syntheses and focus group interviews with key populations, and these components are reported elsewhere (Refol et al., 2025). **Supplemental Material Appendix 1** describes these complementary research activities.

3. Role of the funding source

There was no involvement by the study funders.

Table 1
Characteristics of expert panel ($N = 129$).

	N (%)
Profession	
Community member / person with lived experience	12 (9.3)
Community organizer, leader, or advocate	18 (14.0)
Non-profit employee	12 (9.3)
Public servant / government official	4 (3.1)
Researcher	109 (84.5)
Educator	45 (34.9)
Healthcare service provider	12 (9.3)
Mental health professional	25 (19.4)
Other	5 (3.9)
Expertise/Experience - Communities	
Indigenous People	10 (7.8)
General Population	109 (84.5)
Racialized People	24 (18.6)
Migrants, Immigrants, and Refugees	23 (17.8)
People with Disabilities	28 (21.7)
People with Chronic Diseases	41 (31.8)
Sexual and Gender Minorities	27 (20.9)
Other	16 (12.4)
Expertise/Experience - World Regions	
Africa	5 (3.9)
Asia	13 (10.1)
Caribbean	1 (0.8)
Central America	2 (1.6)
Europe	51 (39.5)
North America	90 (69.8)
Oceania	14 (10.9)
South America	4 (3.1)
Other	4 (3.1)
Expertise/Experience - Age Groups	
0 – 13 Children	18 (14.0)
14 - 18 Youth	42 (32.6)
19 - 30 Young Adults	90 (69.8)
31 - 54 Middle-Aged Adults	81 (62.8)
55 + Seniors	71 (55.0)
Expertise/Experience - Topics	
Community Building (e.g., development, maintenance, advocacy, connectedness, facilitation)	31 (24.0)
Equity, Diversity and Inclusion (e.g., Health Equity, Social Justice, Diversity) Intergroup relationships, Social Inclusion	29 (22.5)
Loneliness / Social Isolation	82 (63.6)
Social Connection, Peer Relationships, Friendship	79 (61.2)
Social Support	53 (41.1)
Social Skills	20 (15.5)
Social Prescribing	15 (11.6)
Dating, Sexuality, and Intimate Relationships	17 (13.2)
Social Anxiety / Social Phobia	16 (12.4)
Suicide Ideation / Suicide Attempts / Suicide	13 (10.1)
Social Health Interventions or Implementation science	42 (32.6)
Solitude and positive effects of being alone	11 (8.5)
Other	27 (20.9)

4. Results

4.1. Participants

A description of our Delphi panel participants is provided in [Table 1](#). In summary, the panel of 129 experts included a diverse mix of community members, organizers, non-profit employees, government officials, researchers, educators, healthcare providers, and mental health professionals. Their expertise spanned various communities (e.g., Indigenous, racialized, migrants, people with disabilities, sexual and gender minorities, people with chronic diseases), world regions (e.g., Africa, Asia, Europe, North America), gender and age groups (children to seniors). Key topical areas of expertise covered included community building, equity and inclusion, loneliness, social connection, support, skills, prescribing, relationships, anxiety, suicide, and social wellbeing interventions.

Round 1 of the Delphi study focused on brainstorming potential guidelines and the principles that should guide the development process. Detailed descriptions of results from Round 1 are provided in **Supplemental Materials Appendix 2**. In summary, Round 1 of the Delphi study identified guiding principles for: (1) the development and dissemination of the guidelines (e.g., accessibility, flexibility, evidence-based, universal design), (2) the format and content of the guidelines (e.g., concise, easy to understand, achievable), (3) how the guidelines should be framed (e.g., as advice, in relation to loneliness and health, adaptable to cultural and personal circumstance), and (4) specific recommendations for individuals and communities.

The suggested individual guidelines provided recommendations related to: (a) prioritizing social health, (b) developing healthy social cognition, (c) investing wisely in one's social life, (d) building resilient social networks, (e) getting enough social connection, (f) meeting people and making friends, (g) deepening and maintaining connections, (h) getting the most from digital connections, (i) being a proactive socializer and example to others, and (j) navigating inevitable social challenges.

The community guidelines provided recommendations focused on: (a) community leaders promoting social connection, (b) improving accessibility and inclusion, (c) incorporating social wellbeing into policy and governance, (d) investing in community infrastructure, (e) supporting community programs, and (f) conducting monitoring, evaluation and research. Specific statements reflecting these themes were generated by synthesizing the insights provided across experts.

Round 2 of the Delphi study asked participants to provide initial rankings of the importance of the expert-generated guidelines and development principles. Detailed descriptions of results from Round 2 are provided in **Supplemental Materials Appendix 3**. In brief, Round 2 focused on gathering participant ratings of the importance and agreement with expert-generated guidelines and development principles, yielding nuanced insights across individual, collective, and systemic dimensions of social connection. Experts strongly supported principles emphasizing flexibility, inclusivity, and evidence-based approaches. For example, 96.2 % agreed or strongly agreed that the guidelines need to be based on research evidence that was representative and generalizable to target populations, 87.2 % agreed or strongly agreed that the guidelines needed to be flexible enough to cater to the diverse social needs of individuals, and a similar proportion (83.3 %) agreed that the recommendations needed to be tailored to different target populations and cultures. Conversely, a large proportion also endorsed the idea that the guidelines should be able to be applied universally, with 57.7 % agreeing or strongly agreeing. Other features, such as the use of numeric goals (e.g., number of friends), received lesser support with only 51.2 % endorsing these, and 61.5 % endorsing qualitative and flexible recommendations over specific numeric goals. Despite some points of disagreement, the majority of principles and specific proposed guidelines had high levels of support, allowing us to move forward with synthesizing this information to generate a draft set of guidelines.

Following a review of the rankings and qualitative comments

generated in round 2, we developed a framework for the guidelines, which synthesized statements into a draft set of 12 guidelines, consisting of six guidelines for individuals and six guidelines for communities. This was accomplished by consolidating thematically related recommendations that were highly supportive and rated as important. In doing so, we balanced the high level of support across statements with the need for a concise set of public health guidance that could be feasibly implemented. [Table 2](#) provides an overview of these draft guidelines as they were presented to the experts.

The third round of the Delphi study assessed the degree of expert consensus for the generated guidelines. As shown in [Table 3](#), for each of the guidelines, we assessed consensus among all experts who rated each guideline, all experts rating any guideline in the survey (since individuals could choose not to rate an item if they felt they lacked expertise), and all experts who indicated that they had “high” self-rated domain-specific knowledge on each specific guideline. All 12 guidelines achieved a high degree of consensus in each of these groups, meeting group-specific criteria of >75 % support, >80 % support, and 85 % support among the groups, respectively. Additional detailed results from Round 3 are provided in **Supplemental Materials Appendix 4**.

Given high support of the drafted guidelines, Round 4 focused on refining the language and structure of the guidelines through qualitative focus group interviews. Key “additional considerations” were also generated from the focus group interviews. Detailed results from Round 4 are provided in **Supplemental Materials Appendix 5**. Summarizing this content, [Table 4](#) provides the final guidelines generated through this process and a list of the additional considerations generated. Final guidelines with the complete content for each additional consideration is provided online at www.socialconnectionguidelines.org. The website also includes: (a) a rationale for each guideline, (b) an internal working group's assessment of the quality of evidence and potential limitations of each guideline, (c) supporting case studies, (d) the texts of evidence briefs that informed the development of the guidelines, (e) research article summaries for key studies reviewed as part of the guideline development process, and (f) a list of guideline-specific implementation resources to help with their uptake.

5. Discussion

In this study, we developed public health guidelines specific to social connection by leveraging the expertise of a diverse global panel through a structured Delphi methodology. The guidelines were developed to provide a concise framework for both individual-level and community-level action. Importantly, the decision to adopt a social-ecological framing emerged early from Round 1 brainstorming responses, where experts emphasized the need for guidelines that addressed both individual and broader societal levels. On the basis of this input, the research team adopted a social-ecological lens as a foundational organizing principle. Draft guidelines were only developed after Round 2 and formally presented to participants in Round 3 for rating and refinement. Within this framework, individual-level guidelines emphasize the importance of prioritizing social connection, cultivating a positive social outlook, building diverse and meaningful networks, and judiciously using technology to enhance relationships. Community-level recommendations focus on fostering supportive environments and policies, improving accessibility and inclusion, and establishing robust measurement frameworks to monitor social health. These recommendations are designed as public health guidelines—intended to be accessible to individuals while also providing a framework for service providers, policymakers, and community leaders. Similar to guidelines on nutrition or physical activity, they are not solely targeted at individuals but instead aim to inform a whole-of-society approach to fostering social wellbeing. This broad orientation also guided our selection of experts from across research, practice, government, and community sectors. High-level consensus of support for these guidelines and their adoption of flexible, universal design principles aims to ensure their utility in

Table 2
Draft guidelines.

Draft Guideline
Guideline 1: Make social connection a priority throughout your life. • Social connection is a basic human need for people of all ages. • Try to be thoughtful of how much and what types of social connection you need. • Meeting your social needs often requires effort and intentionality. Guideline 2: Cultivate a positive social outlook and sense of self. • Many individuals experience thoughts and feelings that stop them from engaging with others, including fear of rejection, social anxiety, self-doubt, and comparison to others. • Try to counter these thoughts and feelings by practicing strategies such as (a) fostering self-compassion, (b) grounding your beliefs in objective facts, (c) experimenting with taking gradual steps outside your comfort zone, and (d) trying to assume the best with respect to how others feel about you. • Additionally, rather than thinking of time spent alone as a negative reflection on you or your social life, try to value this time an opportunity for solitude, a time to recharge, and a chance to meet your personal wellness needs. • If you find it difficult to overcome your thoughts and feelings, seek support from others, including trained mental health professionals who can help you reach your social health goals. Guideline 3: Build a strong social network. • Strive to develop a close circle of several trusted friends and family, rather than relying on just one person to meet all your social needs. • Work to have positive and meaningful connections in all areas of your life, including at work, school, home, and in your neighbourhood communities. Guideline 4: Get enough social connection, but invest your social energy where it counts. • Individuals need regular and frequent social interaction. • Strive for near-daily social interactions, even if brief. • Spend at least a few hours every week socializing with your close friends and family. • Prioritize close, rewarding relationships, but don't neglect social interaction with acquaintances or even strangers. Guideline 5: Develop new relationships and deepen the ones you have. • Take initiative to improve your social life and the social life of those in your communities. • Be open to new connections and build the skills and strategies needed to make new friends. • Try to strengthen your relationships by fostering reciprocity, openness, and trust with others. • Learn and practice the social skills needed to navigate social challenges, including good communication, compassion, and boundary-setting to maintain healthy relationships. Guideline 6: Use technology as a tool for social connection. • Seek out and prioritize opportunities for face-to-face, in-person social interactions with others. • Actively use technologies and platforms that benefit and strengthen your relationships with others, such as those that help you organize social activities or interact meaningfully with others while you are apart. • Limit passive use of social media and other technologies when they distract from social interactions, monopolize your time and attention, or negatively impact how you feel about yourself or others. Guideline 7: Promote awareness of the importance of social connection. • Play an active role in promoting widespread awareness of the importance of social connection. Guideline 8: Foster healthy social and emotional development • Develop and implement age and community-appropriate social and emotional education activities, curriculum, resources, and other tailored supports to support the development of social health. Guideline 9: Make social health a priority in all policies and practices • Make social connection a top public health and policy priority. • Consider the social health impacts of policy decisions and review existing policies to assess how they can foster social connection. • Make substantial investments in social health programming and organizations that support social connection. Guideline 10: Build communities for connection and invest in social health programming • Ensure that the built and natural environment is suitable for encouraging, facilitating, and sustaining social connections. • Implement programming that maximizes social use of the built and natural environment thereby providing opportunities for social connection and interaction within communities. Guideline 11: Improve accessibility and inclusion for all people. • Ensure that all spaces and activities are equitably accessible for all community members. • Ensure that tailored activities and programs are available for people of different ages, abilities, socioeconomic classes, cultural backgrounds, and with different social and recreational interests. • Actively seek to eliminate stigma, discrimination, and violence against marginalized communities. Guideline 12: Measure progress towards improving social health. • Develop measurement frameworks for understanding the social well-being of your community. • Measure the social health of your community and monitor progress towards improvements in social health. • Identify the best, evidence-based strategies for promoting social health. • Prioritizing strategies that reduce disparities in the social health of equity-seeking communities.

diverse social contexts.

While our guidelines are highly novel, we can compare the guidelines we've developed to other guidelines for social connection and health behaviour in several respects: First, we note that our guidelines adopt a multi-level social-ecological approach – providing guidelines to individuals and communities – which has been advocated for decades in guideline development for other contexts but not routinely implemented

in the construction of most national guidelines [19]. By adopting this advice, we anticipate that the social connections guidelines will have a larger impact than what is typically seen from general practice-based lifestyle change interventions [20].

Second, and specific to social connection guidelines, the guidelines developed in this study reveal similarities and differences with other proposed social connection guidelines, such as those offered by Holt-

Table 3
Expert endorsement of draft guidelines.

Draft Guideline	Experts Rating the Item	All Experts Completing the Survey	Experts with High Self-Rated Domain Knowledge	Number of Comments Received (Total Word Count of Comments)
Make social connection a priority throughout your life	97.5	96.3	98.1	35 (1126 words)
Cultivate a positive social outlook and sense of self.	83.8	77.5	86.7	44 (2601 words)
Build a strong social network.	96.0	93.5	97.8	27 (852 words)
Get enough social connection, but invest your social energy where it counts.	88.9	92.1	90.2	37 (1720 words)
Develop new relationships and deepen the ones you have.	95.8	88.5	97.6	31 (1316 words)
Use technology as a tool for social connection.	88.2	78.9	86.2	40 (1599 words)
Promote awareness of the importance of social connection.	98.6	92.2	97.2	29 (665 words)
Foster healthy social and emotional development	94.4	87.2	92.0	27 (736 words)
Make social health a priority in all policies and practices	98.6	90.1	100.0	20 (467 words)
Build communities for connection and invest in social health programming	97.3	91.0	96.6	21 (409 words)
Improve accessibility and inclusion for all people.	100.0	97.4	100.0	16 (604 words)
Measure progress towards improving social health.	94.4	87.2	96.8	26 (721 words)

Lunstad (2023). The individual guidelines, in particular, align with these prior efforts by emphasizing the importance of social connection and providing guidance on the structure, function, and quality of relationships—a tripartite framework originally advanced by Holt-Lunstad, Robles, and Sbarra (2017) [3]. However, unlike these other emerging guidelines, our recommended guidelines do not suggest specific numerical targets (e.g., a specific number of friends, or a specific amount of social time each week). This decision follows what we observe with other national guidelines for nutrition, exercise, and sleep which focus on core. Within our framework, guidelines such as ‘build networks,’ ‘develop new relationships,’ and ‘deepen existing ones’ may appear overlapping, but in fact reflect distinct dimensions of social wellbeing. Specifically, ‘building networks’ emphasizes the structural diversity of social ties across family, friends, coworkers, and community members; ‘developing new relationships’ highlights network sufficiency and size; and ‘deepening existing ones’ emphasizes relational quality. While these domains are interrelated, they are not interchangeable—individuals may, for example, have many diverse ties but low-quality relationships, or few close ties but strong supportive quality. Unlike other established guidelines, however, our recommendations intentionally avoid prescribing specific numerical targets (e.g., a specific number of friends or amount of social time per week). This decision follows the precedent of other national guidelines on nutrition, exercise, and sleep, which emphasize universal core principles (e.g., movement, mindful eating) rather than rigid quantitative thresholds, allowing for broad adaptability across contexts [21,22].

Our community guidelines can also be compared to existing frameworks for promoting social wellbeing, such as the U.S. Surgeon General’s Advisory on Social Connection [13], which focuses on the importance of community infrastructure, development of pro-connection policies and practices, and education across a wide variety of stakeholder groups. Similar to those guidelines, we propose a whole-of-society approach to support individuals to lead healthy social lives, acknowledging that a traditional neoliberal public health approach that places responsibilities solely on individuals is neither feasible nor effective [23].

In addition to aligning with the content of previously developed guidelines and frameworks, we also note significant external validity of our expert-developed guidelines to related evidence. Indeed, we previously published nearly 50 evidence syntheses on key topics and questions related to the guidelines (see **Supplemental Material Appendix 1** for list of evidence briefs) providing a synthesized evidence base for our guidelines. While the robustness of individual studies may be limited,

congruence in the overall messages of the existing literature aligns with our guidelines.

5.1. Implications for policy and practice

Although we acknowledge that the development of our guidelines is a starting point for public health responses to loneliness and social isolation, the high level of support they achieved and their congruence with existing authoritative sources underscore their potential value. Application of these guidelines in policy, healthcare, organizational, and educational settings stands to raise the profile of social wellbeing as a legitimate determinant of health on par with traditional determinants such as nutrition and exercise. By emphasizing both individual and community efforts, the guidelines aim to drive cultural shifts that norm and support social connections and provide capacity for meaningful and sustained social connections.

5.2. Limitations and future research

Our study has some limitations. First, because participation in this study was voluntary, our Delphi panel may not fully represent the full range of professional or cultural perspectives. This self-selection limits the generalizability of our findings and underscores the need for further validation of the guidelines among both professionals and the general public. Such challenges are common in Delphi studies, which often rely on smaller or less representative panels; however, our study did include one of the larger and more diverse panels to date. Importantly, while the guidelines were designed to capture universal principles of human social connection—and social connectedness is indeed a basic evolutionary need—the ways in which connection is fostered vary across cultures and contexts. Accordingly, future work should focus on adapting these guidelines to local environments to ensure their relevance and applicability. This is especially important, noting the over-representation of experts from North America and Europe, which reflects broader global inequities in social connection scholarship. Although loneliness and disconnection are increasingly recognized as global concerns, including in low- and middle-income countries, the empirical evidence base is disproportionately from high-income countries. The World Health Organization has initiated processes to develop guidelines for diverse contexts, and future research should evaluate and adapt our recommendations beyond high-income countries. Second, expert opinion represents a low standard of evidence. Partially addressing this limitation, we note that we have conducted a variety of evidence reviews and

Table 4

Final guidelines and summary statements for each additional consideration.

Final Guideline Individual Guidelines	Additional Considerations
Make social connection a priority throughout your life. Social connection is a basic human need for people of all ages, but meeting your social needs often requires time, effort, and intentionality. • Be thoughtful and aware of how much and what types of social connection you're getting and whether you're getting enough social connection. • While recognizing that relationships are not always easy, strive to pursue meaningful, satisfying, and positive connections with others. Cultivate social confidence in yourself and others. Sometimes how we feel about ourselves and others makes social connection intimidating and difficult. These feelings are normal and can be overcome. • Be open to social interaction and connecting with others. • Start by taking small steps and look for manageable ways to expand your capacity. • Be compassionate with yourself and others. • Know that it often takes time to improve your social wellbeing. • If you find it hard to overcome negative thoughts and feelings about yourself or about your social interactions with others, get support, including, if possible, from trained mental health professionals. Build a strong social network with a variety of kinds of relationships. A strong social network includes a variety of social connections across different domains of your life. This includes interactions of varying depth and quality as well as a variety of different types of relationships in different areas of your life. • Diversify where you get social connection from; don't rely on just one person to meet all of your social needs. • Work to have positive and meaningful connections in different parts of your life, including at work or school, home and neighbourhood, and with other groups you are connected to or share things in common with. • Be open to meeting new people and socializing with others, even those you may not currently know or be close to. This includes connecting with people who might be different from you. Invest in getting enough social connection. Individuals need regular and frequent social interaction. This includes spending time with those we're close to as well as with other people we are less familiar with. • Seek out opportunities for near-daily social interactions, even if brief. • If possible, spend quality time every week socializing with the people closest to you or with those you would like to become closer to. • Prioritize meaningful and satisfying relationships over superficial and less fulfilling ones, but remain open to new and deepening relationships. Maintain and deepen your relationships with others. Positive relationships are among the most important contributors to our health and happiness. However, all relationships require care and attention to thrive. • Strive to develop a close circle of supportive people you can trust and count on. • Care for others by fostering mutually-beneficial, intimate, open, and trusting relationships. • Pay attention and be responsive to the social needs of others. • Learn and practice the social skills needed to navigate social challenges and maintain healthy relationships, including good communication, compassion, and boundary-setting. Seek out face-to-face interactions and use technology wisely. Technology is an important tool for facilitating social interactions, but it is important to ensure that we do not replace face-to-face connections with online ones. • Ensure you have plenty of opportunities for face-to-face, in-person social interactions with others.	• Loneliness and isolation harm health; social connection is as vital as diet and exercise. • Recognize diverse relational needs like belonging, support, and authenticity. • Balance daily interactions and deep connections for social fulfillment. • Reflect on needs; use withdrawal as a cue to reconnect. • Respect varied social needs while prioritizing wellbeing. • Build inclusive connections through empathy and systemic change. • Seek positive groups for trust, belonging, and support. • Know when to nurture, reset, or end relationships. • Focus on actions you can control to improve social wellbeing. • Set SMART goals to grow and balance social connections. • Build authentic connections by challenging assumptions and fostering understanding. • Recognize how experiences and biases shape social cognition; create positive environments. • Overcome negative beliefs with small steps, strengths, and growing confidence. • Balance self-reflection with empathy and mindfulness to enhance resilience. • Embrace alone time as growth and relaxation, reducing dependence on others. • Approach social time with warmth, gratitude, and positivity to deepen connections. • Break rumination by seeking support from friends, professionals, or communities. • Seek support to navigate challenges, break rumination, and build social confidence. • Strengthen connections through authenticity, boundaries, and balanced relationships. • Counter negative self-perceptions with compassion and gradual social engagement. • Build confidence by challenging negativity, visualizing positivity, and staying open. • Social networks vary; most rely on a close circle along with broader, less frequent ties. • Value quality over quantity, focusing on meaningful relationships for wellbeing. • Build resilience with strong one-on-one bonds and cohesive group connections. • Cultivate diverse ties to meet varied needs and enhance social wellbeing. • Seek diverse connections to foster empathy, learning, and equality. • Strengthen your network through shared activities, warmth, and consistent effort. • Show openness with approachable gestures, light conversation, and follow-ups. • Keep your network strong by engaging current ties, rekindling old ones, and forming new ones. • Balance deep relationships with casual connections for emotional and social fulfillment. • Maintain connections through regular touchpoints and openness to renewal. • Balance casual and meaningful interactions to feel connected and fulfilled. • Invest in a trusted inner circle for support, joy, and resilience. • Expand your network by engaging with acquaintances and community groups. • Prioritize quality over quantity in connections for greater wellbeing. • Use solitude for growth, reflection, and recharging. • Set realistic goals for balancing casual and meaningful social interactions. • Incorporate easy, daily opportunities for spontaneous connections. • Create shared experiences through enjoyable, routine activities that build bonds and memories. • Foster strong relationships with active listening, clear communication, and emotional safety. • Build trust through reciprocal self-disclosure and empathetic listening. • Strengthen bonds by expressing gratitude, celebrating successes, and showing kindness. • Maintain healthy relationships by sharing tasks fairly and addressing imbalances. • Integrate relationships into broader networks to foster community and connection. • Focus on positives and address challenges constructively to build trust and intimacy. • Create inclusive, supportive communities through collaboration and shared initiatives. • Develop social skills like communication, empathy, and problem-solving for better interactions. • Navigate challenges with communication, empathy, and boundaries, seeking support when needed. • Build close friendships through shared experiences, consistency, and mutual support. • Prioritize uplifting, reciprocal relationships and deprioritize draining ones. • Combine face-to-face depth with digital convenience for balanced connections. • Use technology mindfully to complement relationships and assess its impact. • Build digital literacy, avoid echo chambers, and balance online and offline interactions. • Balance hybrid work with in-person bonding and flexible virtual connections. • Prioritize active tech use, set boundaries, and focus on meaningful interactions.

(continued on next page)

Table 4 (continued)

Final Guideline Individual Guidelines	Additional Considerations
- When using technology, use tools and platforms that benefit and strengthen your relationships with others, such as those that help you organize social activities or interact meaningfully with others while you are apart. - Limit passive use of technologies when they reduce your social contact with others, distract from social interactions, monopolize your time and attention, or negatively impact how you feel about yourself or others.	- Resolve digital conflicts with empathy, clarity, and thoughtful tools. - Avoid comparisons and echo chambers on social media; prioritize genuine connections.
Community Guidelines	
Promote awareness of the importance of social connection. All organizations play an important role in facilitating social interactions and creating norms that make social connections easier. - Play an active role in promoting widespread awareness of the importance of social connection through health promotion, community dialogues, professional education programs, policy-making and other efforts. Foster healthy social and emotional development. Strong emotional and social skills foster health and happiness. To achieve these skills, communities should facilitate and support individuals at all stages of life to refine their social skills and meet their varied social needs. - Develop and implement age and community-appropriate social and emotional education activities, curriculum, resources, and other tailored supports that empower individuals and communities to connect. Make social connection a priority in policies and practices. Policies and practices across all layers of governance play an important role in shaping social wellbeing. Many policy areas that are not immediately relevant to social connection or health can have profound consequences on social and community wellbeing. - Consider the social impacts of policy decisions and review existing policies, and integral collaboration between policy domains, to assess how they can foster or inhibit social connection. - Invest in social programming and organizations that support social connection. Design environments for connection and invest in social events, activities, and programs. Built, natural, and social environments play an important role in facilitating social interactions. - Ensure that the built and natural environments are suitable for encouraging, facilitating, and sustaining social connections. - Implement programming that maximizes social use of the built and natural environment thereby providing opportunities for social connection and interaction within and even across communities. Improve accessibility and inclusion for all people. While all human beings need social connection, people in every community can experience barriers to accessibility and inclusion. These barriers can create unique vulnerabilities to loneliness, isolation, and disconnection. Communities must work together to remove these barriers. - Ensure that all spaces and activities are equitably accessible for all community members. - Ensure that tailored activities and programs are available for people of different ages, abilities, socioeconomic classes, cultural backgrounds, language groups, and with different social and recreational interests. - Actively seek to eliminate stigma, discrimination, and violence. - Advance reconciliation, equity, diversity, and inclusion. Measure and make progress towards improving social wellbeing. Achieving social wellbeing at the individual and population levels requires ongoing investments in monitoring and improving social connection. - Allocate funding and resources for research and evaluation on social connectedness. - Develop measurement frameworks for understanding community social wellbeing. - Measure social wellbeing at a community-level and monitor progress towards improvements in social connection. - Identify the best evidence-based strategies for identifying and supporting communities and individuals at risk for social disconnection. - Coordinate across sectors to prioritize strategies that reduce disparities in social wellbeing equity-seeking communities.	- Promote connection through education, community programs, media, and workplace initiatives. - Align efforts with community values to foster inclusion and social cohesion. - Celebrate diversity with inclusive events that honour heritage and build community. - Encourage a whole-of-society approach to create inclusive, resilient spaces. - Promote lifelong social and emotional learning with adaptable, accessible programs. - Strengthen inclusivity by fostering intergenerational and intercultural empathy and learning. - Incorporate cultural traditions into social-emotional education for meaningful engagement. - Integrate social connection into policies for community resilience and wellbeing. - Treat social connection as a key health factor in all policies. - Create inclusive policies in diverse settings to foster community belonging. - Build connection by embedding social metrics and work-life balance initiatives. - Ensure resources support programs that strengthen social ties. - Remove barriers through affordable housing, fair wages, and accessible healthcare. - Engage communities directly in creating and implementing relevant policies. - Promote equity by supporting economic stability, work-life balance, and social services. - Host events that strengthen group bonds and unite diverse communities. - Design environments to encourage interaction and community wellbeing. - Use inclusive programming to promote engagement and build connections. - Ensure programs adapt to evolving needs and diverse social groups. - Involve communities in planning to enhance ownership and civic pride. - Encourage volunteerism and local leadership to sustain projects and networks. - Offer inclusive programming tailored to diverse groups and interests. - Empower marginalized groups to lead and maintain culturally relevant initiatives. - Provide safe, judgment-free spaces to support and connect marginalized communities. - Enhance digital access and literacy to empower vulnerable populations. - Reduce stigma and bullying through education and awareness campaigns. - Use a cross-sector approach to reduce social disconnection and isolation. - Empower communities with accessible resources and active involvement in research. - Add social health indicators to healthcare screenings to address unmet social needs. - Incorporate social health indicators into public health tracking to guide policies. - Use mixed-methods research to understand social wellbeing comprehensively. - Shift to inclusive research methods to address marginalized communities' needs. - Apply the OCAP framework to ensure ethical research with Indigenous communities. - Engage communities in participatory research to promote equity and mutual respect.

consultations with key populations (reported on in Refol et al., 2025) in order to inform our guideline development process. Third, Delphi studies risk an over-emphasis on consensus among experts. Real-world validation of the guidelines to determine their effectiveness is needed. For example, investigating how the public perceives these guidelines may be important, as previous research (e.g., in the context of the

COVID-19 pandemic) has shown that health recommendations can be contentious in the public sphere [24,25]. Similarly, while studies examining the effectiveness of health promotion guidelines has shown that they can promote behaviour consistent with recommendations, this consistency varies by behaviour and population, with generally small to moderate effect sizes [26,27]. Future work should empirically evaluate

the uptake and health impacts of these guidelines. Such evaluation may require modernization of social wellbeing surveillance frameworks, as current national surveys capture some, but not all, relevant dimensions of social connection. Rigorous testing of adherence, health outcomes, and contextual moderators across diverse populations will be essential to inform ongoing refinement. Longitudinal multi-site research can support the evaluation and the potential efficacy of these and other public health guidelines [28,29]. Without such research, it will be difficult to ascertain whether our guidelines are ultimately effective in achieving their goals of promoting social connectedness [30]. Thus, the Delphi method used here provides a starting point for the validation of public health guidelines, but continued research is needed. In particular, it is important to note that while the guidelines articulate core principles for promoting social wellbeing, our Delphi study stopped short of specifying validated measures or operational metrics. As with guidelines in nutrition and exercise, future research should focus on developing reliable and culturally sensitive indicators that can be used for monitoring, evaluation, and implementation.

6. Conclusion

In conclusion, the development of public health guidelines for social connection represents a significant advancement in addressing the pervasive contemporary issues of loneliness and social isolation. These guidelines offer actionable strategies for individuals, communities, and policymakers to foster social well-being, grounded in expert consensus and inclusive of diverse perspectives. While the guidelines mark an important milestone, their ultimate success hinges on ongoing research, inclusive and equitable implementation, and thoughtful refinement.

Data sharing

Our team welcomes collaborative research. Data for this study will not be made publicly available as they contain semi-identifiable information that could compromise research participant privacy.

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Declaration of competing interest

The authors declare no competing interests.

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Supplementary materials

Supplementary material associated with this article can be found, in the online version, at [doi:10.1016/j.healthpol.2025.105452](https://doi.org/10.1016/j.healthpol.2025.105452).

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